



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Mental health patient management: Emergency department

Reference #: FSFH-HW-POL-0052

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Emergency Department; Mental Health Service	Medical, Nursing, Allied Health, Administration, Facilities Management

1. Introduction

Timely access to specialist mental health assessment and care can minimise prolonged hospital admission and/or inappropriate discharge into the community. The Mental Health Liaison Service – Emergency Department (MHLS-ED) aims to facilitate timely assessment and patient flow from ED by rapidly stabilising patients and planning their ongoing care.

This policy and procedure outlines the clinical roles and standards required for triaging, assessing, managing and discharging patients who present to the ED with suspected or confirmed mental health issues /psychiatric morbidity.

2. Terminology

Absent person	Any voluntary mental health patient whose whereabouts should be known but cannot be ascertained after diligent efforts to locate and/or contact them have been made by staff. However, the patient's life or health is not believed to be in danger, and they are not believed to be at risk of harming others.
Missing person	The Chief Psychiatrist defines a 'missing person' as 'Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.' 'Harm' includes harm to self, harm to others and harm to property, such as arson.

The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

Absent without leave (AWOL)	When an involuntary person is absent without approved leave (AWOL) from a hospital or other place (Mental Health Act 2014 s.97).
Duty of care	A legal and professional obligation to take reasonable and responsible steps to not cause foreseeable harm to another person and to safeguard others.
Patient centred care	Care based on the principles of personhood, individualised care and empowerment. When providing care to people it is necessary to consider the whole person within their social context, recognising their unique needs, experiences, values and preferences and to support self-determination in decision making.
Person centred cultural care	Is about ensuring the patient journey is culturally safe and reliable. In the Aboriginal context it focuses on respecting cultural differences, partnering with patients and their families; the effective responsiveness of service delivery and a commitment to ensuring cultural security within the hospital setting.
Recovery oriented	Practice that supports people in taking responsibility for their own recovery and well-being and pursuing their life goals. In any setting, when clinicians are recognising and responding to a person, recovery-oriented care involves sharing responsibility for safety with consumers to the greatest extent possible, creating opportunities for the person to regain their self-control and supporting their autonomy to pursue their life goals
Trauma informed care	Many people who access mental health services have experienced trauma in their lives. Trauma-informed care assists in creating physical, psychological and emotional safety for individuals who may be suicidal or are seeking care.
Emergency short stay unit (ESSU)	Inpatient unit located within the Emergency Department (ED) for patients who are expected to be discharged home within 24 hours of admission.
Emergency Department Information System (EDIS)	Computer information system utilised within ED for tracking of patient information and clinical information.

Psychiatric Online Service Information System (PSOLIS)	Patient database used by WA public Mental Health services.
Emergency psychiatric treatment (EPT)	Treatment required to save a person's life or prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, nominated person, the parent, or guardian of a child and any guardian or enduring guardian of an adult.
Transfer of care	When the clinical responsibility for providing treatment, care and support is moved from one service provider to another. This may occur within a service or between different services.

3. Policy

- The Mental Health Liaison Service (MHLS) - Emergency Department (ED) will provide a 24-hour a day liaison service to ED clinicians to support them in managing mental health patients.
- The MHLS consists of a Consultant Psychiatrist, Registrar, Mental Health Liaison Nurse (MHLN) and Peer Worker.
- The ED and Mental Health Service (MHS) will collaboratively assess and manage people presenting to the ED with mental health issues and psychiatric conditions.
- Care for patients presenting to the ED with mental health issues will be:
 - recovery oriented,
 - person centred,
 - trauma informed,
 - culturally competent and
 - developmentally appropriate.
- All patients with a primary presenting issue related to a psychiatric disorder must be discussed with an ED Registrar or Consultant.
- Patients will be referred to the MHLS-ED via EDIS referral. Prior to completing the referral, the ED clinician will discuss the patient with the MHLS-ED team to confirm the referral is appropriate.
- The ED clinician will clearly communicate the nature of the request, either:
 - check history for known mental health patient and/or

- provide assessment/advice.
- Where an active mental health patient presents to ED with a mental health related issue/complaint the patient must at a minimum be discussed with the MHLS-ED prior to discharge.
- Medical treatment does not need to be completed prior to review by the MHLN or MHLS-ED. Patients who are alert, orientated, coherent and able to concentrate, are suitable for psychiatric assessment and the MHLN may commence review of a patient without request if warranted.
- The MHLN can recommend to ED medical staff that a patient remain in hospital for further assessment or management by psychiatry but does not have authority to discharge a patient from ED without ED medical involvement in the decision.
- Involvement of the MHLN is not a substitute for medical assessment and management.
- The MHS only assumes care of the patient when explicitly agreed and accepted. Until this time the patient remains under the clinical governance of the ED team.
- The following patients referred to MHLS-ED for review must be reviewed by a Psychiatric Registrar and escalated to the Consultant Psychiatrist before leaving the ED:
 - patients < 16 years
 - perinatal/post-natal
 - with clear and imminent risk of suicide / self-harm or harm to others
 - medically complex
 - referred to the ED for mental health input by a Medical Practitioner e.g. General Practitioner (GP)
 - subject to the Mental Health Act (MHA) 2014.
 - planning discharge for patients > 65 years
 - presentations to ED within 7 days of discharge from any mental health inpatient unit
- All patients less than 18 years of age referred to the MHLS-ED must be discussed with a Consultant Psychiatrist within 8 hours of admission to the ED.
- A parent, carer or Nurse special must remain with any patient under 18 years waiting in the ED for an inpatient admission.
- Patients assessed as requiring an inpatient admission will be transferred to the appropriate service as soon as a bed is available.
- As cultural differences can result in variable mental health presentations the Interpreter service will be used to facilitate patient assessment when required.

- With the patients consent referral to the Aboriginal Health Liaison Service may be made during business hours to support the patient and family:
 - The Aboriginal Health Liaison Service may consult with the FH Aboriginal Mental Health Liaison Service
- Where preferential for patient care mandatory mental health documentation may be commenced by the MHLS-ED clinician in ED and completed on admission.

4. Collaborating with family/significant others

The essential role of a patient's family/significant others in providing collateral information regarding the patient's mental state will be recognised and respected.

- The MHLS-ED will identify and contact the patient's family / significant other / PSP's wherever possible to obtain collateral history and support them to participate in assessment and treatment planning.
- Collateral information may include early warning signs of change in a person's mental health, management strategies that have worked in the past and the impact of medication.
- Wherever possible a patient's family /significant other/PSP will be actively included in safety and discharge planning.
- Provide family/significant others of patients who have made a suicide attempt or are experiencing thoughts of suicide information on:
 - how to escalate care if the patient deteriorates
 - safety planning
 - information on referrals and aftercare
 - available mental health crisis services
- Inform the family/significant other/PSP of relevant notifiable events. Refer to MHA 2014 Part 9 – Notifiable events Division 1 & 2.
- All communication with the patient's family/significant other/PSP will be documented in the patient's medical record.
- If the patient is less than 18years and under the protection and care of the Department of Communities - Child Protection and Family Support, they are the legal guardian and must be contacted for collateral information. The Department must be informed of treatment decisions and notifiable events.
- Staff from the Department of Communities – Child protection and Family Support will attend the ED to provide support to a patient under their protection and care or may authorise a carer or family member to provide this support.

5. Roles and responsibilities

5.1. Community Treatment Teams (CTT)

- When a CTT determines an active community mental health patient requires transfer to ED, the CTT Clinician will:
 - Provide direct telephone handover to ED staff, ideally medical to medical via ED medical shift coordinator.
 - Contact the MHLN to discuss reason for presentation and admission.
 - Attend ED where possible with the patient for face-to-face clinical handover.
 - Complete the MH Assessment Form and Risk Assessment and Management Plan (RAMP) in webPSOLIS and upload in the digital medical record.
 - Document treatment plan.
 - Provide ED staff with family/significant other/personal support person contact details.

5.2. Mental Health Liaison Nurse

- Is the main point of contact, communication, and coordination between the MHLS-ED and the ED team.
- Oversees and coordinates the activity of the MHLS-ED, the mental health assessment and the monitoring and care of patients in ED who have been referred to the MHLS-ED and triages all referrals.
- Sources collaborative history and patient information from referrers, other health professionals, patient's family/significant other and PSOLIS
- Updates mental health information in EDIS.
- Assists with management of acute distress and/or behavioural disturbances.
- May refer patients who are transiting ED on their way to other wards to the MH - Consultation Liaison team for later review, however responsibility for referral lies with the medical team receiving the patient.

5.3. ED Triage Nurse

- Identifies and assesses patients whose presentation may be affected by psychiatric, behavioural or emotional problems alongside other health problems.
- Assigns ED Australasian Triage Score (urgency).
- Considers the assessed risk, degree of patient distress/agitation and likelihood that patient will leave before being assessed "Did not wait" (DNW). Makes a clinical decision regarding:

- the most appropriate location for the patient to wait (waiting room or inside the ED), recognising a waiting room is not an appropriate area to leave a person experiencing an acute phase of mental illness.
- need to inform/discuss the patient with the MHLS-ED.
- the requirement for Security staff presence.
- the requirement for immediate escalation to MHLS-ED and/or escalation to the Medical or Nursing Shift Coordinator for additional resources.
- Consider the patients cultural and spiritual concerns:
 - Consult with the Aboriginal Health Liaison Officer/Aboriginal Mental Health Liaison to address cultural needs when indicated.
- Ongoing liaison with the MHLN regarding decisions as required.

5.4. ED Nursing Shift Coordinator

- Ensures patients presenting with MH conditions are streamed to an appropriate area of the ED for the care they require.
- In consultation with MHLS-ED identifies need for a behavioural management plan to be instigated:
 - Consult with the Aboriginal Health Liaison Officer/Aboriginal Mental Health Liaison to address cultural needs when indicated.
- Liaises with the Area Lead, Capacity and Access Service and MHLS-ED to arrange an appropriate bed if an admission is likely.
- Informs the MHLN of significant changes to the patient's presentation or level of risk and liaise with the MHLN regarding care of those patients waiting for extended times to be seen by MHLS-ED
- Liaises with the ESSU Lead Nurse and Area Medical Lead regarding patients to be transferred to the ESSU to await a MH bed or MH assessment / management prior to transfer.
- If a nursing 'special' has been requested:
 - Ensures request has been documented as per the Specialising and observation: Provision of special for patients in non-psychiatric clinical areas policy and procedure and rationale has been provided by the MHLS-ED
 - Escalates request to the Nurse Unit Manager/Senior Registered Nurse on duty for approval by Nurse Director.

5.5. ED Treating Doctor

- Completes medical assessment and treatment to exclude possible medical causes for mental health symptoms.

- Conducts psychiatric assessment with risk assessment for patients with a suspected psychiatric disorder.
- Discusses patient with the MHLS-ED prior to completing referral.
- Completes a referral to the MHLS-ED team in EDIS with details of patient including risk concerns.

5.6. ED Medical Shift Coordinator

- Receives all external MH referrals.
- Acts as a gatekeeper to the ED by confirming that all other reasonable avenues of care have been explored prior to sending the patient to ED.
- Enters new MH referral details into the EDIS 'Patient Expects' list informing the Nursing Shift Coordinator and MHLN of the referral.
- Liaises with Nursing staff if preparations need to be made for receiving a distressed patient.

5.7. Psychiatric Registrar / Health Service Medical Practitioner (HSMP)

- As the delegate of the ED/Duty Consultant Psychiatrist the Registrar is responsible for psychiatric assessment and management of patients, and liaison with the patient's family/carer personal support person who have been referred to the MHLS-ED.
- Collaborates/liaises with the MHLN to:
 - determine individual workloads and prioritise between themselves.
 - update each other regarding management of allocated caseloads.
 - periodically update the ED team regarding assessments and management plans.
 - prioritise between ED and the Mental Health Assessment Unit to ensure flow between the two areas.
- Contacts the ED/Duty Consultant Psychiatrist when:
 - clinical advice is required.
 - a patient's mental state deteriorates.
 - there are unresolved differences in perspective between the MHLN or another colleague on aspects of the treatment plan or with clinicians from another specialty regarding patient flow issues.
 - there is insufficient staff in ED to cope with current or anticipated demand.
- Seeks support from Mental Health Assessment Unit Registrar and Intern and/or ED/Duty Consultant Psychiatrist at times of high demand or when the MHLN is absent to avoid developing a backlog of cases.

5.8. ED Consultant Psychiatrist

- The MHLS-ED Consultant Psychiatrist:
 - Reviews/provides advice on the management of patients escalated by the Psychiatric Registrar
 - Following assessment of patients less than 16 years discusses the management plan with the Child and Adolescent Mental Health Service (CAMHS) on call Consultant Psychiatrist where required.
 - Undertakes advisory and supportive work for the MHLS-ED in the ED and provides supervision for Psychiatric Registrar/HSMPs in ED.
 - Manages the flow of patients from ED into definitive care.
 - Where necessary, advocates for patients with the Consultants of other specialties and at other MH sites.
 - Contacts the Metropolitan Patient Flow Coordinator (08:00 to 17:00 hours) or the Patient Flow Medical Director (after hours) to advocate for patients whose transition to other services has been delayed.
 - Takes an active role in resolution of disagreements regarding clinical issues when escalated by ED staff, the Psychiatric Registrar or the MHLN.

6. Patient assessment and treatment planning

6.1. Mental Health assessment

- MHLS-ED clinicians will complete a comprehensive mental health assessment including a mental state examination and document on the SSCD MH Triage Form in webPSOLIS and uploads to the medical record.
- Patients under 16 years may be admitted to ED overnight to facilitate Consultant Psychiatrist review or direct referral to CAMHS.
- Mental Health patients under 16 years assessed as requiring immediate CAMHS expertise and support are referred to CAMHS Crisis Connect. Referrals To CAMHS Crisis Connect require a completed:
 - Mental Health Triage
 - Risk Assessment and Management Plan (RAMP)
 - Paediatric social work review if appropriate.
- Referrals are:
 - emailed to CAMHS Crisis Connect
 - followed up with a phone call to Perth Childrens Hospital Bed flow to schedule a time for assessment.
 - dedicated teleconferencing equipment is in the MHLS-ED Office.

6.2. Risk assessment.

- MHLS-ED Clinicians will complete a RAMP in webPSOLIS and upload it to the medical record.
- The overall mental health assessment and RAMP will assist the Clinician in discussion with appropriate mental health colleagues to determine the treatment plan e.g. admission to the Mental Health Unit, transfer to another MHS or discharge from the ED with appropriate community follow up.
- Transfer to other MHSs will be undertaken in accordance with the [Mental Health Bed Access, Capacity and Escalation Statewide policy](#)

6.3. Transfer of care

- The MHLS-ED will provide all patients reviewed and discharged from ED with a clear mental health discharge plan and document the plan in the patient's medical record including referrals or follow up arrangements.
- The MHLS-ED will collaborate with the patient and family/significant other in safety planning for patients with identified psychiatric risk including suicide:
 - Safety planning may include identification of warning signs, internal and external coping skills, scenario planning and counselling on access to lethal means.
- Provide the patient and carer with written advice including contact details of the relevant community MHS and timelines for follow up by the MHS wherever possible and 24 hours MH emergency contacts.
- Where an active FSFHG Mental Health Service patient is seen in the ED the MHLS-ED will:
 - enter the patients details on the FSH MH ED Liaison Contact sheet detailing demographics (including Aboriginality), presenting problem and plan/outcome.
 - Email the Contact sheet daily to relevant staff
- Complete an e-Referral to the FH Assessment and Treatment Team (ATT) Triage/Urgent Care where follow up is required:
 - include as much detail as possible on purpose of referral and risk concerns.
 - ensure patient demographics are up to date.
 - follow up with call to Triage/Urgent Care when able.
 - Triage/Urgent Care will access relevant details from the medical record as required.
- Where a patient is admitted to a general inpatient bed but requires further mental health assessment the MHLN-ED will:

- request the admitting specialty submit an e-Referral to the inpatient MHLS and/or
- liaise directly with the inpatient MHLN to advise them of the patient's admission.
- If the patient's clinical presentation necessitates admission to an inpatient general bed without review by the MHLS-ED, the ED Clinician can refer directly to the inpatient MHLS via e-Referral.

6.4. Patients not referred to Psychiatry

- The ED Medical Officer is responsible for making the decision regarding the patient's disposition.
- If the patient is discharged home the responsible ED Medical Officer must ensure that:
 - a management plan including details for adequate, safe and appropriate follow-up is in place.
 - with the patient's consent, a reasonable attempt is made to ensure the patient's next of kin/carer is aware of the management plan.
 - the patient and where relevant next of kin/carer will be provided with written information on MH emergency contact details and any follow up arrangements.
 - a physical assessment is documented and there is a management plan for any active medical problems.
 - the above-mentioned points are clearly documented with the disposition destination confirmed.
 - clear discharge documentation is sent to the treating team, GP and/or other agencies to which the patient has been referred.
- Patients may be referred to ATT for follow up without being assessed by the MHLS-ED with the reason for non-assessment before discharge documented.

6.5. Management of behavioural disturbance

- Most ED patients who present with behavioural disturbance and unpredictable behaviour do not require specialist psychiatric referral or recourse to the MHA 2014.
- On notification that a patient with a known or suspected psychiatric condition(s) is referred to ED, the clinician receiving the information makes an entry in the EDIS "Patient Expects" screen including the following details:
 - estimated time of arrival
 - diagnosis

- MHA 2014 status
- known risks.
- Inform the MHLN as soon as possible.
- On provision of three approved patient identifiers the MHLN searches for risk alerts, treatment history and/or management plan on PSOLIS.
- ED staff will liaise with the MHLN and other involved staff to ensure dissemination of these details to all relevant staff members.
- Medical causes for behavioural disturbances and/or deterioration in a patient's mental state will always be considered and appropriate treatment provided e.g. pain, delirium.
- Cultural and spiritual concerns must be considered where appropriate.
- When available, Resus Room 1 which has dual egress will be utilised to contain and assess any patient who is potentially aggressive.
- If risks are identified a Patient Risk Alert Sheet is completed based on information from PSOLIS (see [Appendix 1](#)). The sheet is forwarded to:
 - FSH Triage Department to record the information in EDIS for ongoing identification of risk related to that patient.
 - FSH Safety and Incident Management Services (SIMS) if the risk is assessed as imminent.

All staff must immediately activate a "Code Black – Personal Threat" whenever they feel personally threatened.

- Refer to:
 - Acute Severe Behavioural Disturbance (ASBD) Management in the Emergency Department
 - FSFHG "Management of Agitation in Older Adults with Dementia or Delirium" guideline
 - FSFHG "Alcohol and Other Drugs: management of addiction, toxicity and withdrawal" policy and procedure
 - Restraint in non-mental health areas

7. Searching mental health patients

- Patients identified as at risk of self-harm or suicide should be searched for objects or medication that may be used to self-harm.
- Searches for the clinical safety of the patient should be conducted by clinical staff. SIMS staff can assist with searches.

- SIMS staff are required to receive consent to search a person. If consent is not given due to medical reasons, permission must be received from the Senior Doctor.
- Under the MHA 2014 a referred person who presents to the ED or who has been detained may be searched if it is suspected that they have in their possession items, such as a weapon or drugs, which may compromise the health and safety of themselves or others.
- While the MHA 2014 provides search powers, a voluntary search with the patient's consent is preferable.
- Any substance found that cannot readily be identified or is identified as either illicit or otherwise controlled substance must be reported immediately to SIMS for collection and appropriate storage.
- Refer to:
 - Mental Health Act 2014
 - [SMHS Searches in mental health](#) policy.
 - SIMS Metal Detector Serviceability and usage work instruction
 - SIMS Hospital user property management procedure

8. Absent, Missing or Absent Without Leave patients.

8.1. Absconding risk assessment

- If it is determined by ED staff that a patient is at risk of absconding the MHLN is informed as soon as possible, and the Area Lead enters "Review Before Discharge" onto EDIS.
- The MHLN undertakes a PSOLIS search for risk alerts and treatment history.
- Factors that may contribute to the risk of patients absconding include:
 - previous absent/ missing/AWOL
 - extended length of stay in ED
 - emotional distress / agitation
 - intoxication / withdrawal
 - deliberate self-harm/suicidal ideation
 - young people, especially males
 - older people with cognitive disorders
 - Aboriginal and Torres Strait Islander patients
 - culturally/linguistically diverse patients
 - refugees.
- If absconding risks are identified a Patient Risk Alert Sheet is completed based on information from PSOLIS and clinical assessment.
- The sheet is forwarded to Triage for entry in EDIS and sending to SIMS.

- Patients assessed as at risk of absconding are:
 - transferred from the waiting room into a waiting area inside the ED.
 - considered for 1:1 supervision/special.
 - engage family/significant other
 - flagged with the Aboriginal Health Liaison Service within business hours where appropriate
 - prioritised for assessment by the MHLS-ED Team.
- The referring Clinician will engage, briefly review and provide an indication of waiting time to patients triaged as semi-urgent or non-urgent.
- If an “at risk” patient or patient with a “Review Before Discharge” status is interrupted whilst trying to abscond and redirection is ineffective contact SIMS and request that the patient be stopped from leaving whenever possible.

8.2. Absconded patients not yet referred or referred and not seen by the MHLS-ED

- If a patient goes missing from the ED, The ED Triage Nurse informs the MHLN of the patient’s departure as soon as possible.
- The MHLN will review all available information in PSOLIS to provide any collateral information to the ED clinical team.
- An immediate assessment must be completed on the risk the individual poses to self and others.
- The assessment will be used to determine if the patient is considered:
 - absent
 - missing
 - AWOL
- This assessment will inform notification and reporting processes and guide the level of urgency of response required:
 - ED staff will seek advice from the MHLS-ED regarding risk and appropriate action as required.
 - Refer to the [SMHS Missing or suspected missing mental health patient’s policy](#)
- The ED clinical team in conjunction with information from the MHLS-ED will determine the course of action. Interventions will be determined by the patient’s assessed level of risk and may include:
 - no further action
 - contacting the patient by phone to assess risks then determine further action.

- contacting the carer or family member by phone to obtain further information then determine further action.
- requesting assessment by the local community Mental Health team
- notifying the WA Police and discuss/recommend interventions to be implemented by WA Police personnel.
- Document the management plan in the DMR and
- Notify missing or AWOL patients in the Datix Clinical Incident Management System (CIMS).

8.3. Patients seen by MHLS-ED or who are absent without leave.

- The MHLN-ED reviews the DMR/PSOLIS and decides on further action.
- Interventions will be determined by the patient's assessed risk level prior to absconding and include:
 - reviewing assessed risk documented on the RAMP.
 - contacting the patient by phone.
 - contacting carer or a family member by phone.
 - informing the ED/Duty Psychiatrist
 - informing the referrer.
 - informing SIMS and request that a search of the campus be undertaken.
- For active Mental Health patients request follow up by:
 - the relevant CTT during working hours
 - the relevant catchment area ATT after hours (where practicable)
- Request WA Police assistance to apprehend and return the patient to ED via Form 7D for assessment if risks are assessed as clear or imminent and contact is not possible.
- Report as a notifiable event when a referred person who has been detained or an involuntary detained patient is absent without leave. Refer to [SMHS notifications in mental health](#) policy.
- Document the process in the patient's medical record and notify in Datix CIMS.

9. Mental Health Act 2014 application in ED

9.1. MHA 2014 forms

All approved forms are available electronically at [Mental Health Act Forms 2014](#).
Forms relevant to ED may include:

Form number	Form Name	Notifiable event - inform PSP
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Form 1A	Referral for examination by Psychiatrist/Revocation of referral by AMHP or medical practitioner) Attachment to Form 1A Information provided by another person in confidence	Notify revocation only
Form 1B	Variation of referral	
Form 3A	Detention Order	Notify
Form 3B	Continuation of Detention	
Form 3C	Continuation of Detention to enable further examination by a psychiatrist.	Notify
Form 3D	Order of authorising reception & detention in an authorised hospital for further examination	Notify
Form 3E	Order that person cannot continue to be detained (must be used by Psychiatrist to revoke 1A)	Notify
Form 4A	Transport Order/Revocation of Transport Order	Notify
Form 4B	Extension of Transport Order	
Form 6B	Inpatient Treatment Order in Authorised Hospital (ITO)/Revocation of ITO	Notify
Form 7D	Apprehension and Return Order/ Revocation of Apprehension and Return Order	
Form 8A	Record of Search and Seizure	
Form 9A	Record of Emergency Psychiatric Treatment	
Form 9B	Report to the Chief Psychiatrist about provision of urgent non-psychiatric treatment	Notify

9.2. Form 1A – Referral for Examination by a Psychiatrist

- A referral for examination by a psychiatrist can only be made by a Medical Practitioner or Authorised Mental Health Practitioner (AMHP) who has assessed the patient face to face within the preceding 48 hours.

- A Medical Practitioner or AHMP may refer a person for examination by a psychiatrist if they deem that the person satisfies the following criteria:
 - the person is experiencing a mental illness for which they require treatment.
 - as a result of the mental illness there is a significant risk to the person's health or safety, or that of another person, or a significant risk of serious harm to the person or another person.
 - the person does not demonstrate that they have the capacity to make treatment decisions.
 - there is no less restrictive way to provide the required treatment.
- If Form 1A is written to the ED, the patient is immediately received when the referred person enters the ED.
 - Complete the bottom of the Form 1A to identify the commencement of the 24-hour period during which the person must be examined by a psychiatrist.
- If Form 1A is completed while the person is in the ED and the place of examination is the ED, the 24-hour period in which the person must be examined by a psychiatrist commences immediately.
- If the Form 1A is completed external to the ED and the person's condition is such that the person needs to be detained a Form 3A Detention order can be made. This is valid for 24 hours from when the order is made.
 - Immediately the referred person is received at the ED they are detained to enable the Psychiatrist to examine them and the Form 3A has no further validity.
- If the person is made a referred person while in the ED and the place of psychiatric examination is the ED, the person is automatically detained. Form 3A Detention Order is not required.
- Once the patient is reviewed the Consultant Psychiatrist will make one of the following orders:
 - Form 3E – that the person cannot continue to be detained.
 - Form 3D - authorising the person's reception and detention at an authorised hospital to enable an examination by a psychiatrist at the authorised hospital.
 - Form 6B - an inpatient treatment order in a general hospital
 - Form 5A – community treatment order

9.3. Management of Form 1A or at-risk patients

- The MHLS-ED staff member must inform the Nurse Area Lead position immediately when:
 - a patient is assessed as clear and imminent risk, or

- the patient is referred under the MHA 2014 and must not be allowed to leave ED.
- The Nurse Area Lead will ensure that a “Review Before Discharge” flag is added to EDIS (see [Appendix 2](#) for process).
- Any patient requesting to leave with “REVIEW” noted in the EDIS tracking screen is to be detained and the MHLS-ED notified immediately.

10. Emergency psychiatric treatment (EPT)

- At any time during the referral process a referred person may require treatment such as medication to manage their distress, symptoms or behaviours.
- Where a referred person provides informed consent to the treatment offered, then the treatment is not deemed to be EPT.
- EPT provides more specific statutory protection for staff than duty of care/common law. Refer to SMHS [Emergency psychiatric treatment](#) policy
- EPT may be given to voluntary patients (who have not been referred) or referred patients (MHA 2014 Form 1A) who become behaviourally disturbed and meet the criteria for EPT, specifically where treatment is needed to:
 - save the person’s life or
 - prevent the patient from behaving in a way that is likely to result in physical injury to the person or another person.
- Where a referred patient refuses treatment or is unable to consent because of their mental state and they meet the criteria for EPT treatment can be administered without their consent.
- The MHLS-ED or Duty Psychiatrist will advise on the requirement and application of EPT.
- In the majority of EPT cases the treatment provided is psychotropic medication
- The Medical Practitioner who provides EPT must, as soon as is practicable, record that EPT was provided and complete a [Form 9A](#) as per the requirements of MHA 2014 (s.204). A copy of the form must be provided to the:
 - Office Chief Psychiatrist monitoring team.
 - referred patient.

11. Compliance and Performance monitoring

The Service Directors of Services 4 and 5 will be responsible for monitoring compliance with this policy and procedure. Compliance will be monitored via:

- clinical incident trending related to mental health patients AWOL from the ED as reported through Datix CIMS
- patient feedback and/or complaints
- percentage of MH ED attendances with a length of episode greater than 24 hours

12. Related policy documents

- WA Health:
 - [Recognising and Responding to Acute Deterioration Policy](#)
 - [Safety Planning for mental health consumers](#)
 - [Safety Planning procedures for mental health consumers](#)
 - [Mental Health Consumers who are missing or absent without leave](#)
 - [Requesting Road Based transport for Mental Health patients Subject to Transport Orders Policy](#)
 - [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)
- SMHS:
 - [Missing or suspected missing mental health patients](#)
 - [Emergency psychiatric treatment](#)
 - [Discharge against medical advice](#)
 - [Notifications in mental health](#)
- FSFHG:
 - Clinical risk assessment and management (FSFH-HW-POL-0017)
 - Clinical care of people who may be suicidal (FSFH-HW-POL-0037)
 - Specialising and observation: Provision of special for patients in non-psychiatric clinical areas (FSHGP11)
 - Clinical Handover (FSFH-HW-POL-0035)
 - Clinical documentation (FSFH-HW-POL-0039)
 - FSH - Code Black

13. Other related documents

[Western Australia Mental Health Act 2014](#)

[Helping Minds Practical guide for working with Carers of people with a Mental illness](#)

14. Related standards

- NSQHS Standards:
 - Clinical Governance
 - Partnering with consumers
 - Comprehensive care
 - Communicating for safety
 - Recognising and responding to acute deterioration
- Chief Psychiatrists Standards for Clinical Care:
 - Assessment
 - Risk assessment and planning
 - Transfer of Care

15. Bibliography

[Australasian College of Emergency Medicine. Access to care for patients with acute mental and behavioural conditions. V3. December 2022.](#)

[Royal College of Emergency Medicine. Mental Health in Emergency Departments. A toolkit for improving care. Last revised April 2023.](#)

16. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	06/2016	Nurse Director, Mental Health		FSH EC	06/2019
2	03/2018	Nurse Director, Mental Health	NMEC	FSFHG Policy Committee	03/2021
3	06/2019	Nurse Director, Service 5		FSFHG Policy Committee	03/2021
4	06/2020	Clinical Nurse Specialist, Mental Health	Service 5 SQR Committee	FSFHG Policy Committee	06/2022
5	05/2022	Clinical Nurse Specialist, Mental Health	Service 5 SQR Committee NMPGC	FSFHG Policy Committee	09/2025
6	10/2024	Clinical Nurse Specialist, Clinical Improvement	Service 5 SQR Committee NMPGC	FSFHG Policy Committee	12/2026

17. Appendices

17.1. Appendix 1: Patient Risk Alert Sheet

PATIENT RISK ALERT SHEET – FOR ED TRIAGE & SIMS

1. Affix patient sticker

Does the patient has any of the following (circle):	
History of violence or aggression	YES / NO
History of harm to others, including hospital staff	YES / NO
History of harm to self	YES / NO
History of Absconding	YES / NO
Comments (e.g. treating doctor, relevant management details, nursing special while in ED):	

Signature
Designation

Print name
Date

Scan and email directly to: fsh.simsoperations@health.wa.gov.au

18. Appendix 2: Entering “Review Before Discharge” onto EDIS

The MHLSED to notify the Area Lead as soon as the decision is made on ‘not to leave’ and identify the patients as ‘REVIEW’ on the EDIS Nursing column/palate. This is easily observed on the tracker screen so all staff are aware of this decision and to highlight the patient is required to be reviewed prior to leaving. The Area Lead will communicate this to all staff in the area.

ED Nurses		
.MEDICAL TRIAGE MEDICAL TRIAGE	.REVIEW BEFORE DISCHARGE .	ADA

Tracking Screen as at M															
Surname	Fore Name	Status	Alert	Sex	Age	Tr Cat	Arrival Time	LOS	Location	Senior Doctor	Treating Doctor	Nurse	Stream	Presenting Problem / Diagnosis	Bed Ready / Bed Request
JKLSDAF	SDJFKLA	T		F	88	1	14:06	0:34	ABAY					DRUG / ALCOHOL USE	
TEST	PATIENT	T		F	20	2	14:03	0:37	ABAY					GASTROINTESTINAL	
ECLAIR	CHOCOLA	T		F	37	4	+ 13:57	168.43	SECL			REVIEW		NEUROLOGICAL - FEELS F/	